

Surgery for stomach cancer

You may have surgery to remove a stomach cancer. Or if cancer is stopping food passing into the stomach, you may have surgery to help keep the stomach open.

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Surgery to remove the stomach

Surgery for [stomach cancer](#) is done by specialist surgeons. It is not available in all hospitals, so you may need to travel to a hospital further away to have the operation.

The operation you have depends on where the cancer is in the stomach and its size. You may have:

- partial gastrectomy, where part of the stomach is removed
- total gastrectomy, where all of the stomach is removed.

The surgeon also takes away an area of healthy tissue around the cancer. This is to try to make sure all the cancer cells are gone. The area of healthy tissue is called a margin.

They also remove nearby [lymph nodes](#) and the omentum. The omentum is the fatty tissue that covers the stomach and the front of the bowel.

Even when the cancer has spread outside the stomach to the surrounding area, it may still be possible to remove the cancer. If the cancer has spread, the surgeon may remove parts of some [nearby organs](#). This depends on the position of the cancer and how far it has spread. This may include:

- the lower part of the oesophagus (the tube that connects the mouth to the stomach)
- the upper part of the small bowel (duodenum)
- the spleen or part of the pancreas.

This is major surgery, and you may not be well enough to have it. Talk to your surgeon about the benefits and risks of this operation before [making a decision](#).

Most people need treatment with [chemotherapy](#) as well as surgery. Sometimes, surgery is the only treatment that is needed. This is usually when stomach cancer is diagnosed at the earliest possible stage. Or it may be because you are not well enough to have both chemotherapy and surgery.

Gastrectomy

Depending on the position and size of the cancer, it may be possible to keep part of the stomach. This is called a partial gastrectomy. Having this operation makes eating easier after surgery. This is sometimes called a subtotal gastrectomy.

A total gastrectomy is when the surgeon removes the whole stomach. It is usually used if the cancer is in the upper or middle part of the stomach.

Partial gastrectomy

If the cancer is in the lower stomach, the surgeon removes the lower part of the stomach. They then reconnect the remaining part of the stomach to the small bowel.

The stomach is usually connected lower down the small bowel than it was before. This is to stop bile (a digestive fluid) and pancreatic juice from going back up the oesophagus. The juices drain from the gallbladder further down into the small bowel. The surgeon sews up the part of the small bowel (duodenum) that used to attach to the lower stomach.

Cancer in the lower part of the stomach

Image: The illustration shows the organs inside the chest and abdomen. A tumour is shown in the lower stomach. Two dotted lines mark the section of the stomach that is removed. One is a third of the way down the stomach. The other is at the point that the stomach joins the duodenum. There is another dotted line at the start of the small bowel where another cut will be made.

The lower stomach is removed

Image: The organs inside the abdomen after the whole stomach has been removed. The lower end of the oesophagus now connects directly to the small bowel, behind the liver. The duodenum is no longer attached to the bottom of the stomach. Instead the top end by the pancreas has been sewn up, and the lower end has been attached to the small bowel.

Total gastrectomy

The surgeon removes the whole stomach. They usually do this operation if the cancer is in the upper or middle part of the stomach.

When the surgeon removes the stomach, they connect a part of the small bowel to the oesophagus. They connect it slightly lower down than where the stomach used to join. This is to stop bile (a digestive fluid) and pancreatic juice from going back up the oesophagus. The juices drain from the gallbladder further down into the small bowel. The surgeon sews up the part of the small bowel (duodenum) that used to attach to the lower stomach.

Surgery to remove the whole stomach

Image: The illustration shows the organs inside the chest and abdomen. A tumour is shown in the middle of the stomach. Two dotted lines mark the section of the stomach that is removed. One is at the point that the oesophagus meets the stomach. The other is at the point that the stomach joins the duodenum. There is another dotted line at the start of the small bowel where another cut will be made

After surgery to remove the whole stomach

Image: The organs inside the abdomen after the whole stomach has been removed. The lower end of the oesophagus now connects directly to the small bowel, behind the liver. The duodenum is no longer attached to the bottom of the stomach. Instead the top end by the pancreas has been sewn up, and the lower end has been attached to the small bowel.

Keyhole surgery (minimally invasive surgery)

This is when the surgeon does the operation through a few small cuts in the tummy (abdomen), rather than 1 large opening. It is sometimes called laparoscopic surgery. The surgeon puts a long, fine tube with a camera and a light on the end through the cuts to see and work inside the body.

For a total gastrectomy, the surgeon makes a few small cuts and one larger cut. They remove the stomach through the larger cut.

Surgeons sometimes used robotic surgery to treat stomach cancer. The surgeon uses controls to move robotic arms that hold the surgical instruments. Having robotic surgery can reduce side effects for some people and they may spend less time in hospital.

Surgery to relieve a blockage in the stomach

Sometimes the cancer causes a blockage that stops food from passing through the stomach. Your cancer doctor may suggest putting a tube called a stent into the stomach. This can help keep the stomach open, so food can pass through it more easily. It can be placed:

- in the opening at the top of the stomach
- at the lower end of the stomach, where it opens into the small bowel.

You will have a local anaesthetic, which is given as a spray to the throat. Or you may have some medicine to make you sleepy (sedation). Sometimes a general anaesthetic may be used. The doctor puts an endoscope down the oesophagus and into the stomach. They can then pass a stent into the area where the blockage is, to allow food to pass through.

Sometimes the surgeon does an operation to bypass the blockage. They do this by making a new connection between the stomach and small bowel. This allows food to pass through a different way.

Surgery or stents can be used to relieve a blockage and symptoms, but will not cure the cancer.

Your doctor or nurse will talk to you about the preparation and recovery for these types of surgery.

Before your operation

Before your operation, you will have tests to make sure you are well enough. These are usually done a few weeks before surgery at a pre-operative assessment clinic. They include tests on the heart and lungs.

Your surgeon and a specialist nurse will talk to you about the operation. You may meet the doctor who gives you the anaesthetic (anaesthetist) when you are at a clinic. Or you may meet when you go into hospital for the operation.

If you think you might need help when you go home after your operation, tell your specialist nurse as soon as possible. For example, tell them if you might need help because you live alone or are a carer for someone else. Your healthcare team can help organise support before you go home.

You usually go into hospital on the morning of your operation. Or you may go in the night before.

The nurses give you special elastic stockings (TED stockings) to wear during and after the operation. These help prevent blood clots in your legs.

Before surgery, it is important to make sure you have all the information you need. The operation can be complex, so you may want to ask questions. Talk to your surgeon or specialist nurse if there is anything you do not understand.

Enhanced recovery programme

Many hospitals now have enhanced recovery programmes. This aims to reduce your time in hospital and speed up your recovery. It also involves you more in your own care. For example, you will get information about exercises you can do to help you get fitter before surgery. You will also get information about exercises to do after your operation.

It also makes sure any arrangements needed for your return home are organised in advance.

Your cancer doctor will tell you if an enhanced recovery programme is suitable for you.

After surgery for stomach cancer

You will probably be cared for in a high-dependency unit for a few days after your operation. You will probably feel quite tired, and may not remember much about the first day or 2 after your operation.

Drips and drains

You may have some drips and drains attached to your body for a few days after surgery. These include the following:

A central venous catheter (CVC or central line)

A [CVC](#) is a thin, flexible tube that is put into a large vein in the neck, upper chest or groin. It can stay in place for up to a week. It is used to give you fluids and medicines until you can eat and drink again. It can also be used to take blood samples without using a needle.

A nasogastric tube

A [nasogastric tube](#) is a fine tube that goes up the nose and down into the stomach or small intestine. It drains fluid so you do not feel sick.

A feeding tube (jejunostomy)

A [feeding tube](#) is a tube that goes into the small bowel through a small cut in the abdomen. It is used to give you food and nutrients until you can eat again.

Abdominal drain

An abdominal drain is a tube that is put into the abdomen to help drain fluid and prevent swelling.

Urinary catheter

A urinary catheter is a tube that is put into the bladder to drain urine into a collecting bag. It can be removed as soon as you can get up and walk around.

Reducing the risk of complications

The nurses will encourage you to get out of bed and move around as soon as possible. This helps reduce the risk of complications after surgery, such as blood clots and infections. The nurses can help you manage your drips and drains while walking.

A physiotherapist or nurse will teach you deep breathing exercises to help keep your lungs clear. They will also show you how to do regular leg movements to prevent blood clots forming in your legs.

A physiotherapist can also show you how to clear your lungs of any fluid that may have built up because of your operation.

Pain

You will probably have some [pain](#) and discomfort after the operation. Your cancer doctor or specialist nurse will explain how your pain will be controlled.

You may have [painkillers](#) in one of the following ways:

Into the space around the spinal cord (epidural)

The painkillers are given through a very fine tube that the surgeon places into your back during surgery. The tube connects to a pump, which gives you a continuous dose of painkillers.

Patient-controlled analgesia (PCA)

A painkiller is given through a pump that you control. This allows you to give yourself an extra dose of pain relief when you need it.

When you no longer need the epidural or PCA, you have painkillers as tablets or liquids.

If you are still in pain, it is important to tell your healthcare team. Mild discomfort or pain in your chest can last for several weeks. You will get some painkillers to take home with you.

Your wound

You will probably have a dressing covering your wound. This will be removed after the first few days.

How long the wound takes to heal depends on the operation you had. The surgeon may have closed your wound with glue or stitches that dissolve and do not need to be removed. If you do not have stitches that dissolve, they are usually removed about 7 to 10 days after your operation.

Tell a nurse or your cancer doctor straight away if your wound becomes:

- hot
- painful
- leaks any fluid.

Eating and drinking after surgery

You will not usually have anything to eat for the first 48 hours after surgery. When you are fully awake, you may have small sips of clear fluids. The amount of fluids you have is slowly increased. After a few days, when you can drink enough, you will start having small amounts of soft foods, and then normal food in smaller portions. This means the new joins made during surgery have some time to heal.

Feeding tube

You will usually go home with your feeding tube still in, to make sure you get enough food and nutrients and do not lose weight. Before leaving hospital, your nurse or dietitian will show you how to use your feeding tube. If you have a carer, they can learn how to use it too. You will meet the dietitian regularly as an outpatient to check how well you are eating. When you are eating and drinking enough, the tube can be removed.

We have more information about [feeding tubes and nutritional support](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our stomach cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Lordick F, Carneiro S, Cascinu T, Fleitas K, Haustermans G, Piessen A, et al. Gastric cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. *Annals of Oncology*. 2022;33(10): 1005–1020. Available from www.doi.org/10.1016/j.annonc.2022.07.004 [accessed July 2023].

National Institute for Care and Health Excellence. Oesophago-gastric cancer: assessment and management in adults NICE guideline [NG83]. 2018. Available from www.nice.org.uk/guidance/ng83 [accessed July 2023].

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help